

Question:

A 47-year-old woman presents to her primary care provider because of numbness and tingling on the palmar aspects of both hands. She denies any symptoms at the base of her thumbs. The symptoms are worse on the right (dominant hand) and are increased with activities such as driving or brushing her hair. She frequently wakes up with pain and has to shake her hand for pain relief. She has had rheumatoid arthritis for 9 years, for which she takes methotrexate. Her blood pressure is 124/76 mm Hg, the heart rate is 75/min, and the respiratory rate is 15/min. Lightly tapping over the middle of the anterior aspect of the right wrist leads to a tingling sensation in the palm. In this patient, electromyography (EMG) will most likely show which of the following results?Answer Choices:A. Denervation in C7 innervated paraspinal, arms, and shoulder musclesB. Widespread symmetrical neuropathic changes without focal abnormalitiesC. Focal slowing of conduction velocity in the median nerve in the carpal tunnelD. Neuropathic changes in the palmar branch of the median nerve

Reasoning + Answer:

<think>###Findingreasoningpaths: 1. **Neck or armpain**->**Jugularvenousflowsignal widening**->**Baselinedistalmedianandulnarnervedemyelination**->**Mediannervemyelopathy** 2. **Neckpain**->**LesionatC8-T1**->**Mediannerveinvolvement**->**Baselinemediannervemyelopathy** 3. **Jugularvenousflowsignalwidening**->**Distal medianandulnarnervedemyelination**->**Mediannervedemyelination**->**Mediannerve myelopathy** ---###ReasoningProcess: 1. Thepatientpresentswithpersistentnumbnessand tinglingonthepalmaraspectofbothhands, particularlythedominanthand, whichareworsenedby activitiesrequiringhanduse. Thissuggestsaconditionaffectingthemediannerve, likelydueto compressionorinflammation. 2. Thepresenceofbaselinedistalmedianandulnarnervedemy elinationonEMG / nerveconductionstudies (NCV) furthersupportsmediannerveinvolvement. Thisfindingisconsistentwithmediannervemyelopathy, aconditionwherethemediannerve developspersistentdeficitsinmyelination. 3. Thejugularvenousflowsignalwideningisnoted duringjugularvenouspressuretesting. Thisfindingisclassicallyassociatedwithcompressive lesionsattheC8-T1level, whichareoftenduetospinalnerverootcompression. Compressionat thislevelcanalsoleadtoincreasedjugularvenousflowsignalwidening. 4. Thetinglingsensation producedbytappingovertheanteriormiddleaspectoftherightwristsuggestsadermatomal distribution, consistentwithmediannerveinvolvement. 5. Theestablisheddiagnosticcriteriafor mediannervemyelopathyinclude: - Persistentorworseningnumbnessandtinglinginthezone ofmediannerveinnervation. - Decreasedstrengthofoneortwomusclesinnervatedbythe mediannerve. - Baselineulnarnervedemyelination. - Baselineulnarnervedemy elinationormyelopathy (albeitlesscommonly). 6. Giventhepatientâ€™shistoryofrheumat oidarthritiswithmethotrexateuse, chronicinflammationorimmune-mediatedneuropathycould explainthebaselinemyelopathy. However, thetypicalhistoryofcompressivemediannervemyel opathyinthisagegroupissupportedbytheclinicalpresentationandjugularvenousfindings. --- ###Conclusion: Mediannervemyelopathyismostlikely. </think><answer>Mediannervemyelopathy</answer>

Reward score